



Dolores Jean Lavins Center for Humanities in Medicine *presents*

THE TEMPEST

MAYO CLINIC ALIX SCHOOL OF MEDICINE • ANNUAL CREATIVE ARTS PUBLICATION

FOREWORD

Every Mayo student knows the story of how the Mayo Clinic was founded: an enormous tempest rolled through a tiny Minnesota town and left it devastated. From the rubble and debris, the seeds of the Mayo Clinic sprouted thanks to our founders, and it grew into the beacon of innovative medical science and excellent patient care it is today. All this—from a single storm.

Medical education is its own kind of tempest, a flurry of facts, figures and feelings. Students from many walks of life are thrust into a hailstorm of pathology and physiology, a squall of emotions from intense patient interactions involving all aspects of human life. All of a sudden, we are surrounded by purpose: discovery, illness, death, birth, healing—and expected to process these while upholding the ever-nebulous ideals of professionalism and maintaining top-notch test scores. It can feel like a whirlwind at times, leaving us hanging on by our fingertips.

The wind screeches and the rain batters—but the tempest subsides, and things grow in its wake. Just as a Midwestern summer tempest helped create a beacon of healthcare, the tempest that is medical school helps us grow into scientists, teachers, advocates, leaders...doctors.

To not be carried away by the swirling chaos, we must remember the things that ground us. This publication was born as a space to create, intended to be fertilizer for seeds sown through experience in souls tilled like soil.

We are proud to present *The Tempest*: a collection of poetry, prose, and visual art meant to foster this growth and to provide students a creative refuge from the downpour, a forum of discourse amongst classmates about the storm we experience together.



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The Dolores Jean Lavins Center for Humanities in Medicine is proud to sponsor this fifth volume of the Mayo Clinic Alix School of Medicine Creative Arts Publication. The mission of the Lavins Center is to support Mayo Clinic's primary value, the needs of the patients come first, by integrating the arts and other expressions of human culture into the healing environment.

In line with this mission, the Center's programs, educational activities and research in the medical humanities serve patients, families, employees, learners and the larger community, uplifting the compassionate delivery of healthcare. Music, dance and theatrical performances, visual art exhibitions, bedside arts programs and lectures all contribute to the healing environment.

In this digital age and beyond, navigating the challenge of medical education

will increasingly rely on competencies in technology, data assessment and management, and the unending quest to understand the mysteries of our own nature. Advances in artificial intelligence will help to make possible advances only dreamt of within the lifespan of our forebears. How those advances are made to work for the flourishing of mankind, however, is a task best nested within the realm of the Medical Humanities. Creativity inherent in the collaborative study of the humanities is not only desirable but necessary to an effective clinician's lifelong commitment to informed critical thought and generous listening in these times. This dedication to a sustainable and truly integrative model of the healing arts and sciences is well represented by students whose work is collected in this volume. We are honored to have the opportunity to help foster original work by these future leaders in the practice of medicine.



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WHEN YOU ARE FAR AWAY

BY AMAL CHEEMA

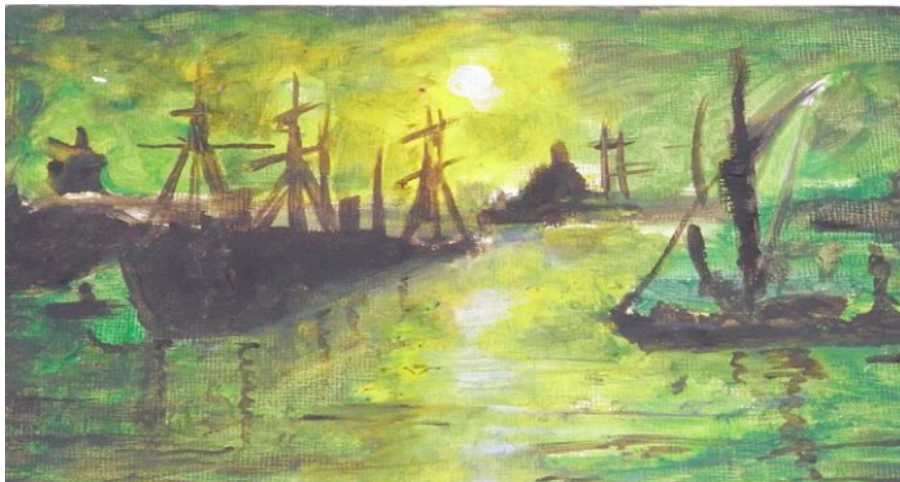
When you are far away,
and thinking of the slow memories
Of falling leaves, and the measured
Swirl of ball gowns in mid-November;
Of candles silenced by gentle movements,
And the quiet moments that fit in the interstices of stolen glances,
and of our subtle flirtations;
Take down those sincere words,
and know that love was the tinted glass behind which we existed,
And all was but a habit –
Of untethered individuals.

November 2014

I wrote *When You Are Far Away* as I was reflecting on what a budding spring love that had faded in autumn might feel like: of two friends who weren't sure of themselves as anything more than ordinary people or strangers. It's centered on a mid-college turmoil in which I found friends who were finding others as they were finding themselves: a moment of identity evolution collectively experienced, individually felt. I drew inspiration for the form and tone from *When You Are Old* by W.B. Yeats, and also, a Taylor Swift song.

FINDING DREAMSCAPE

ACRYLIC PAINTING ON CANVAS
BY DEBORAH "DEBBIE" MSEKELA



Teach me how to glide on the sea's back and ride on the wind's saddle, so that
I too may one day visit the place between sleep and waking, where pixie dust
never turns to ashes.

NARRATIVE

BY TORI RICCELLI

I braid your hair.

Long dark hair flows in waves down your head, a shimmering waterfall of strands swimming through the moving air. Under bare fluorescent lights a single thread floats, tilting up to wave hello to our covered and clogged feet, now dropping once again to a laminated gray floor. A crown of steel encircles your head—a clamp, a prison—gripping sternly to skull, discouraging escape from the loneliest tower.

You are silent and naked. You lie under blue drapes.

The bed is lifted from a floor spilled with solutions and a 'C' is shaved carefully from your thick, youthful scalp. Tubes stick out of each orifice as you are turned to the side and your hair is sponge dyed with the lurid orange fluid.

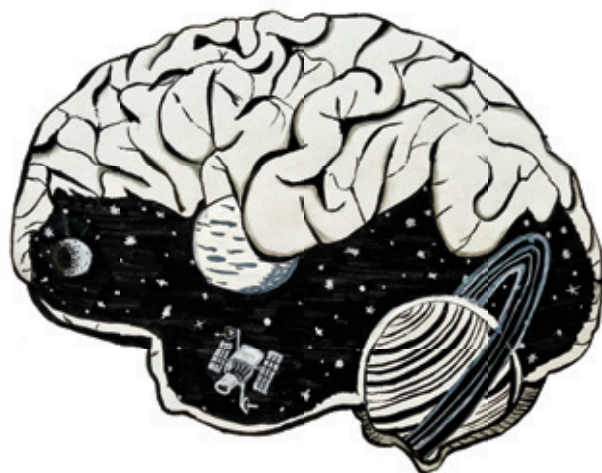
The intern distresses your remaining tresses; a man of steady hands now here in hesitation. Your hair tangles more, angry at his attempts, forming a thick matted shield against the onslaught of his hands. But you must forgive him, for he was never a mother. But I am a daughter in a room full of sons.

So as my mother would pull back my hair in tight braids, so I too pull yours with her discipline and strength. Like my grandmother combed through my hair while she sang, I sing strands to sleep with her calmness and cool. I smooth it all down, for you are my sister, pulling it up and away to know the quiet of your face.

And we cut into your skull.
And we open up your brain.

UNCHARTERED TERRITORY

INK AND PENCIL ON PAPER
BY SHEMONTI HASAN



BADLANDS: A GOOD NAME, SCARES PEOPLE AWAY

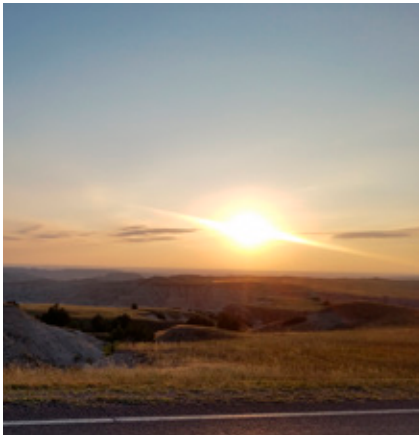
BY SAM ROULEAU

Rock, sand, and prairie grass,
we descend amidst summer's last flare.
Thorned thickets surrender to crumbling
walls
muddy pools hidden under cervices.
Dust paints our lips as
we tumble down the canyon's edge.

Home is the dried riverbed
with gracious flies and skittish mountain
goats as neighbors.
The gray sun lowers
casting purple hues.

We are lost
to the world, except for
merciless winds tearing us apart,
eroded stone listening to our fears,
embraced by the stars.

*September 1, 2018
Badlands National Park, SD*



BADLANDS: CHASING THE SUN & UP CLOSE WITH THIS MAJESTIC BEAST

BY ANJALI PANICKER

ONE WEEK IN SICUNIVERSITY

BY GOHAR MANZAR

Hellbound, MN · “SHEEEEEER INCOMPETENCE!” he thundered ferociously.

The doctor slammed his heavy fist on the table, startling us all as the ominous sound cut through the quiet that crept into the lounge after his outburst.

Worse yet was the silk in his voice, venomous and damning as it draped what he said after. “You were here all night—*all night!* And that patient had a sudden change in his respiratory status at 2 a.m. What possessed you to *even think* that you could just *sit* on getting this chest X-ray instead of getting it *stat?!?*”

The room was too small for us to disperse away from the target of this wrath, and so we all felt lanced right along with her.

“Were you even thinking at all?!” he concluded, his words biting.

If cringing was a sport, we’d be in the Olympics by now.

“I’m sorry,” came her small reply, admirably steady because even I, distant from the fling of venom, found myself shaking with emotion, empathetic.

My dread of this rotation was exponentially increasing, and it had only been two hours since it started. Flittingly, I wondered if I was misguided to be sympathetic to a decision that could have boded badly for the patient. Everything turned out alright, so the ire felt unnecessary at best and tyrannical at worst. But as I walked home that evening, still

reeling from the situation my first morning on SICU, I decided that I was appropriately sympathetic to the human behind those decisions. Yes, she made a questionable judgment call and should have consulted with someone senior, but especially with the outcome, did it merit anything other than firm, but gentle, redirection?

Stopping now at: *Scandalized*. En route to...

Doubtful, MN · I’ve been in the SICU for three days now, and it’s given me plenty to think about.

Our stressful course started out like a runaway train for the team onboard, led by a hotheaded conductor on rusty rails, but our experience collectively improved over the course of the week.

The difficult aspect of it related to several more uncomfortable, *ahem* — “discussions” — on table rounds that wounded resident morale. It gave me plenty to think about with regard to effective and ineffective ways people gain direction and guidance.

As a student, I was always encouraged and treated with kindness. However, it felt unfair to have the protection I enjoy in the “soft coach” — and I know I won’t always be sheltered, especially as I progress in my training. I think I have been shielded from some of the harsh environment that tends to go along with high intensity healthcare such as what takes place in the SICU. Early this week was a flavor of what I might find elsewhere, and I personally come to a conflict as to whether I need to change by acquiring

tougher skin or whether the system needs to change because I shouldn't accept things as they are.

Stopping now at: *Anxious*. En route to...

Hopeful, MN · Maybe it's perception bias, where in the face of dark clouds, you suddenly notice the sun. Despite the challenges this week has brought, there was plenty of good, too.

Learning about the comprehensive focus of the SICU and how things are done there was so informative. I appreciate the good grasp I got on ventilator settings, ABGs, and fluid status. Presenting patients according to a detailed systems review really transformed how I will evaluate and think about critically ill patients in the future. I have grown as a student, a medical professional, and as a person on the SICU.

I did a few dressing changes, placed a couple of i.v. lines, and built many heartwarming relationships with patients and their families. There were many tissues I passed this week and several backs I rubbed. There were many thoughtful conversations about code status, and I was confronted with the harrowing conflict that comes to light with our advanced medical technologies in the face of mortality, when quality of life is sometimes forgotten. I was reminded of how it was our job as a team to remind families about it in a delicate, non-coercive way.

During a very dramatic situation involving severe blood loss in a patient who was DNR/DNI, I could see the patient, his eyes frantic, and his daughter, her gaze stricken as they bonded with his, both change their minds to become DNR only, thereby allowing him to be intubated. Knowing that they were only delaying the inevitable, they clung to the cruel hope for just a

few more weeks in a measure that would require him to be intubated.

He was taken to the interventional radiology (IR) suite as a nurse applied pressure to his abdomen, seated on the bed beside the patient on the gurney that was being carted emergently through sterile white halls and elevators. A huge mass of us followed behind like an exodus, adrenaline coursing in our veins as we raced with IV lines and transfusion products — it is a memory that will glow in my mind forever. I stayed and watched the IR procedure, which was conducted with extraordinary finesse.

Every day brought something different. Stable patients became unstable, and vice versa. Similar to how I functioned on Internal Medicine, I loved performing rounds in the afternoon by myself and getting to know families. Despite the early 6 a.m. start, I ended up staying into 7 p.m. three of my five days on the SICU instead of leaving at 3 p.m. because I really did gain a lot from it and enjoyed it outside of the difficult table rounds early in the week.

Stopping now at: *Wonderment*. En route to...

Fulfillment, MN · The challenging week closed with an incredibly meaningful interaction.

On my way out of the hospital, I ran into the woman whose father was carted off to IR two days earlier. She was waddling out of the patient cafeteria with dinner in her hands, the stress of her father's situation evident in the blank look in her eyes.

I had comforted her during her haphazard, desperate decision to change her father's code status from DNR/DNI to DNR only. She was extremely upset and emotional during his hemorrhage two days prior, as well as during

the code status discussions that followed. Again, she found herself feeling paralyzed as the only child having to make decisions about whether or not to start her father on dialysis. This was yet just another finger in this rapidly crumbling dyke, which wouldn't hold off the hurricane sure to land for very long.

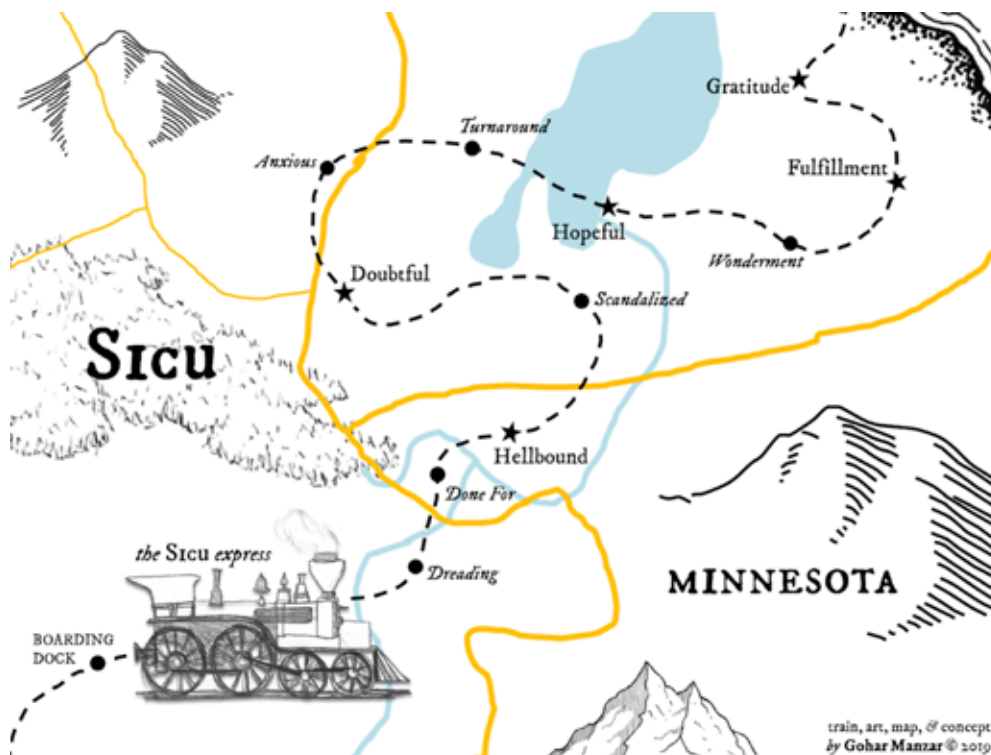
We talked for a while, because she remembered the tissue, my sympathetic words, and my warm touch on her shoulder. On learning that it was my last day on the SICU, she opened her arms to hug me and grew tearful. It made me marvel at the unmatched quality of bonds we cultivate in these tumultuous, fragile times for patients and their families as they teeter at the brink of life and death. In a rotation known across

most medical schools around the country as being one full of grit and formal severity, I encountered the warmest of embraces.

I thought I would taste freedom, that I would be tearing away at the earliest chance to sign off on this much-dreaded rotation, and yet, here I was past dinner on a Friday on an empty stomach but full with fulfillment.

This is the focus of a provider's job on the SICU, this is what healthcare is all about, and this memory is what I will hold onto when I'm that resident one day getting torn into for not ordering a chest X-ray stat.

Last stop, disembark now at: *Gratified*.



AGAINST MEDICAL ADVICE

BY ALYSSA BROWN

In rural Kentucky for surgery rotation, we usually had relaxed call nights at home, which was a ramshackle house across from the hospital. We rarely got called in to a surgery at night. I followed the usual procedure and texted the attending physician, Dr. M at 5pm. This time, he responded with a room number. I could hear his voice wafting down the hallway, so I followed the voice and slid into the patient room silently. Dr. M briefly turned his head to acknowledge that I had found him, but he made no moves to introduce me to the family. The room was dark, and the voices had the unmistakable tinge of fear. I pieced the story together as I stood in the room. The patient was Richard, an 81-year-old male in acute renal failure, which typically is not a surgical problem. He had appeared in the emergency department (ED) eight days prior with a small bowel obstruction. He had refused the surgery and left against medical advice. Richard returned to the ED this afternoon with fulminant renal failure and persistent small bowel obstruction. He had not passed stool in those eight days. Many patients look sick, but he looked close to death's door. He was thin and lying limp in the bed with a faraway look in his eye. His children were gathered around Dr. M. Dr. M used words like *urgent, tonight, and chances of making it through an operation*. Dr. M's phone rang- he told the family he would see them after the operation, then turned and ducked out of the room. I followed him into the hallway, where he took

off clomping in his Dansko clogs.

I could feel the gravity of the situation and the length of the night ahead, so I was glad I had hidden a Kind bar in the pocket of my white coat. As we sat in the lounge, I began to peel off the wrapper and pick at pieces of the granola bar. As if knowing that my mouth was full of granola bar, Dr. M turned and asked what fluids were most appropriate for the patient. I answered wrong with lactated ringers. He berated me for picking an intravenous fluid that would kill the patient. We saw a few patients in the ED, while waiting until the operating room was ready. There was a tension that night. It felt different than the jovial attitude of getting called in for an appendix or a consult for cholecystitis in the middle of the night. I sensed that Dr. M was tense but excited. Dr. M loved the complicated big cases, even though I could tell he was growing tired of call and working so hard at around 60 years old. He loved his patients, and he loved ICU care, yet late at night, I sometimes caught him muttering he was getting too old for nights like this.

Usually, the anesthesia team was joking or playing music when I came in to set out my gloves and gown, but tonight was different. They were a mess. They had intubated the patient and feces came out. Feces were everywhere. It covered the patient's neck and chest. It was splashed all over the floor and over the shoe cover-less feet of the

anesthesia team. I stood paralyzed. I did not know this could even happen, let alone happen tonight. I felt something brush past me. Dr. M immediately jumped into action. He said, “come on, we need to get him open. Clean him up so I can throw a central line in that IJ.”

My nervous excitement juxtaposed with the gravity of the situation. As I entered the operating room once again I heard anesthesia ask if the central line site was clean enough—Dr. M threw back, “This patient needs a central line now, and if it gets infected, so be it, he is going to be lucky as hell to make it off this damn table.” I stood there with my hands crossed, holding each other. I stepped up to the table like I had done many times before. Immediately, the first assist scolded me to re-scrub. Honestly, I was annoyed. I knew I had not touched anything, but I knew better than to question her. She was a stern older woman. She had been around the block and ate medical students for lunch. I charged back out of the OR after impatiently ripping off my gown and gloves. I did not want to miss anything. I came back in just as the abdomen was fully opened. The bowel was distended and grey. It did not look like “happy bowel.” Dr. M told me to follow his hands. We began to run the bowel. Clearly, I was not keeping up at his speed, and I kept tripping over my hands, which I am sure Dr. M noticed, but he did not say anything. We found a small hole, and the obstruction. We pushed on decompressing the bowel. The motion became vaguely soothing. I stole a glance at the clock. It was almost eleven pm. There is serenity from being awake for so long that motions become fluid, almost instinctual.

Dr. M reached for the pool suction as he cut a hole into the stomach. I watched the dark bowel contents fill the large tower was connected to the suction. I kept thinking, the patient is going to feel so much better when we get all these feces out of him.

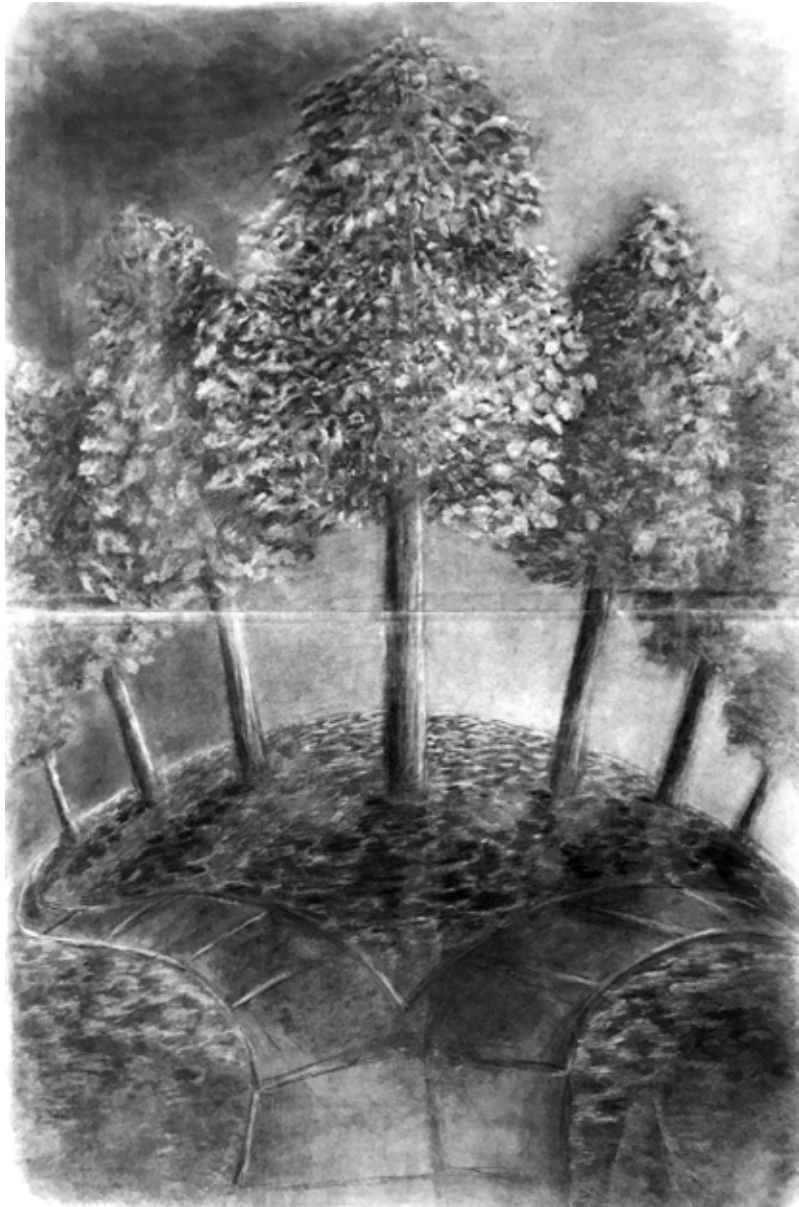
Dr. M slipped the suction out, and asked the anesthesiologist for a range of values – arterial blood gas, base excess, and how fast the fluids were running. At the time, I did not really know what many of those meant, but I knew Dr. M was not happy. After positioning a gastrostomy tube in the stomach, he closed the perforation in the bowel. He then began the difficult task of removing necrotic sections of bowel. After putting in one line of staples across a section of jejunum, he handed me the stapler and told me to run it across the next portion of bowel. I did as I had done many times before, but Dr. M did not like the way I stapled and asked if I was sure it went all the way across, and I said yes; he did not seem to agree, but he moved on. In my head, I thought, “he would usually yell at me for this.” Instead he moved on. This was an ominous sign. I thought to myself, he doesn’t think it will matter. Dr. M quickly interrupted my thoughts and said, “We need to get the hell out of this abdomen.” Dr. M and the first assistant placed the bowel back inside the abdomen. The abdomen seemed too large for the bowel now that the bowel did not look like over-filled sausages. Usually, Dr. M talked a lot in the OR, especially when we were closing, but tonight all we could hear was the beeping of the monitors and the snap of the stapler closing Richard’s abdomen. I took a deep breath because I knew Richard would make it off the table.

We followed Richard to the Intensive Care Unit (ICU). It was midnight, the quiet, inky darkness of rural Kentucky outside. Dr. M looked exhausted. We both stood there just staring at Richard. Occasionally, Dr. M would fiddle with the arterial line. He told me I needed to know how the machine worked by the morning. I knew it was a late night, and there was no way I was going to look anything up when I got home. Still, I nodded. We continued to stare at Richard for an hour at least, standing in silence. I kept wondering what we were looking for. Maybe Dr. M was looking for which way the scale between life and death would tilt. We eventually sat outside of the room to type up notes. He kept snapping at me because I could not, for the life of me, spell cholecystectomy in my sleep-deprived state. We took one last look into the ICU, bed 27, and walked silently down the long hallway back to the locker rooms. He told me that he expected me to hand off the patient in the morning. I nodded with the silent knowledge that both of us would barely sleep between now and the sunrise. I wrote a brief note to my friend, whose patient it would be, and that I would meet her in the morning. I was invested in seeing how this turned out. I knew I wouldn't sleep much that night, worried I would no longer see Richard's name on the list in the morning.

Richard made it through the night. I sighed with some relief that we had tricked death into letting him slide this time. At 11am, I was sitting in the ICU with a different attending physician. I noticed a commotion outside room 27. Dr. M was nowhere to be found in the ICU. Out of the people there, I knew Richard's medical course the best. The family recognized me. As a fresh third year medical student, it was not in my power to relay any meaningful information to the family, but I could give details to the other

medical providers. One nurse said, "He is going to code soon. I'll call Dr. M." I told her I knew where Dr. M was, and could find him faster than a call. I half jogged down the hall towards the operating rooms. I knew he was scheduled for a cholecystectomy in OR 7. I wanted to catch him before he was scrubbed. I still hoped Dr. M could do something to save Richard. I caught him right before he walked into the OR. Dr. M was in his bright orange lead, which I always thought looked slightly comical. Slightly out of breath, I rushed to say that Richard was about to code in the ICU. What Dr. M said next, shocked me. He said, "It was going to happen sooner or later, and I'm not surprised." I fumbled through my words, "Well they were going to call you, but I thought it would be quicker to come tell you in person." He said okay and headed into OR 7. I was too stunned by his reaction to properly form my own, so I turned on my heel and walked back towards the ICU. I didn't know how to handle it. The only person the family knew was me, a lowly medical student, and Dr. M. I wanted Dr. M to be there to talk to the family and support them through what would be a traumatic experience if Richard coded.

I was a boiling pot of anger, frustration, and guilt topped off with helplessness, especially upset because I felt the Dr. M refused to help. When I got back to the ICU, I saw his orange lead coming towards the ICU. I felt slightly more at ease. Dr. M glided towards the chaos outside the room. He parted them, and he talked to the family. He came over later and plainly told me that the family was going to let Richard go peacefully. Dr. M said, "He died by his own hand, he didn't want a surgery until it was too late to help." I suppose in the most basic sense, that's true, but it served as no comfort to me.



CHOICES

CHARCOAL ON PAPER
BY HIBA SAIFUDDIN

EXCLAIM

BY STEPHANIE YOUSSEF

Exclamation point?
Likely the most useless punctuation
mark.
One can easily use other
more sophisticated methods
to elicit a proper reaction from a reader
(diction, literary devices, etc.)
I guess some use it in dialogue.
Despite its rather obvious pointlessness
(oh isn't that kind of witty and purely
accidental),
the appearance of the exclamation
point is rather...
fitting.
It's like an upside down person,
stiff from...
well, I guess the excitement
derived from whatever they are

exclaiming.
Is that what extreme emotion does?
Turn you upside down?
Make you rigid?
Blood rushes to the head.

Well that's not me.
I'm more of a tilde
followed by three spaces
and then a period.
I'm knocked over,
trembling,
and my mind
seems to be out of reach.

Self-portrait: ~ .

SKIES OVER SEGOVIA

BY SYDNEY LARKIN



I took this photo in Spain two weeks before coming to medical school. It represents so many amazing summer memories and the wonderful new journey I was about to embark upon.

HEAVY IS SHE

BY OLUWATOMILONA "TOMI" IFELAYO

Heavy is the head that wears the crochet
Braided strands, beaded twists
Painstakingly planted on the rows of corn
That protect her underlying crop.
Heavy is the hand that waters the obsidian
fields
Depositing moisture in each root
That it may spring forth and grow.
Heavy is the heart that, upon reaping her
luscious harvest,
Dressing it, cleaning it, and decorating it,
Is told at the market that it isn't worthy to be
displayed.
Heavy is she.



STRENGTH & ENERGY, PART II

BY TINA HENDRICKS

I thought I would live by them:
Strength and energy
Energy—I always had
Strength—I trusted to come as required
Until there were neither,
until both were lost.

Who was I without them? And who was I to
become?
A strange mercy to lose
the things on which you most rely.
Not a mercy at all to the broken identity
The shadow of youth's fallibility, revealed all
too soon.

So what's left to be done?
You drink your coffee, as it's there for you
each morning
You journey from one day to the next trying
not to be too hard on yourself
Until the days turn to weeks, and the weeks
to months,
And you see: the manna never faltered in
the wilderness.

Your feet did not swell, your sandals did not
wear out.
And when the strength and energy returned,
you were free from their deception.
Free from relying on yourself.

A MALAWIAN CHILDREN'S WARD

BY KRISTY SESSIONS

The children's ward can be a heartbreaking place.
It is dirty, overcrowded, overwhelming.
Full past capacity with a surplus of sick kids
and paucity of healers.
Diagnostic tests are limited and treatment
shortages are plentiful.
Children are sick.
Doctors overworked.
Mothers anxious.

Walking through the emergency zone is an
assault to the senses.
The smell of fecal matter and too many
bodies is pungent.
The feel of dirty floors and beds, nauseating.
The sound of a mother who has lost a child,
piercing.
The sight of children suffering, heart-
wrenching.

And yet, in this war zone named hospital,
there are hints of beauty.
There are hopes and prayers.
There is companionship amongst caregivers
who chatter as children begin to play
or stand vigil in solidarity as losses are
grieved.
There are countless doses of antibiotics and
compassion.
There are children healing.

In this space where mothers come with little
broken angels on their backs searching for
miracles,
there is hope.
In this ward where children fight for life, there
is healing.
In this land of devastation and sickness,
there is love.
In the midst of it all, there is life.



CLOSE YOUR EYES

BY ALYSSA BROWN

Close your eyes,
trust me,
now picture a surgeon.
What do you see?

Strong,
tall,
light blue scrubs tucked-in,
scrub cap ruffled hair,
a good-looking white male with a strong
jaw?
A savior in scrubs?
A prince riding in on his white horse to save
your life?

Not my mentor,
not me.

Now,
picture my mentor.
Sliding off her four-leaf clover cap,
tucking her bob behind her ears.
Never a hair out of place,
even after long nights on call.
Tired,
but drips grace down the hall,
white sneakers silent.
Hiding the long nights,
hiding the struggle of training.

The not-so-subtle comments—
the OR is no place for you,
for her.

Now picture her.
Under the hot OR lights,
steady graceful hands,
6.5 gloves,
glasses perched on her nose,
quizzing a student
“ten causes of pancreatitis, go”
student scrawling them on the drape
with the purple marking pen.

3.0 Prolene
incision disappears,
maybe it was never even there.
“Needle back”
drapes ruffle off the table.
Gowns ripped off,
tossed into the trash.
The magic moment breaks,
like a bubble popping.
We are back to reality.

But close your eyes,
now tell me who you see.

HERDING CASES

BY GOHAR MANZAR

cataloging patients as only radiation fractions
a disenchanting syndrome of herding cases
they lament the reimbursement lost
and are blind to how behind diagnoses are real faces

for a fat salary, they have the greedy gall to demand
daily clinic trips for six weeks instead of three
to neglect the patients we serve
is decidedly tasteless, cold, and wrong to me

"Gleason 6 reckons blasting because otherwise
we won't see a fifth of the cases in our profession
no matter how the data shows such patients have
the best outcomes under active observation"

when did they lose sight that we are healing sisters and brothers
and not churning a quick buck by zapping masses
when did a life of privilege become so blinding
that they forget what the privilege of living encompasses

In medical school and on my away rotations, I am fortunate to have trained among remarkable individuals who uphold patients at the center of their daily work. This poem was born after recognizing that such values were not represented in an angst-filled discussion by physicians on an anonymous online forum that left me sorely disappointed to read. Those individuals may not have intended for their remarks to have been seen as callous, but they did come across as antithetical to the values nurtured in me in medical school and my life before that. While I may be called naive to the realities of clinical practice, I hope to never become familiar with the disenchantment I saw on the forum. This all being said, the purpose of piece is not to judge others or be holier-than-thou. Rather, it is meant to provide a jolt and a reminder to keep our priorities pure in this rewarding, noble profession of healing others with science and compassion.

MY WEEK WITH THE CHAPLAINS

BY KRIS CHATTERJEE

I chose to become a doctor, in two words, because I saw that all illness brings forth the very stuff of life. What's beautiful, tragic, and altogether meaningful about our human experience reveals itself uniquely in our patient's life drama, often during one of her most vulnerable chapters. Although I'm not actively religious, I do know that whatever this meaningful stuff of life is, it falls also within the domain of religion. This is why I spent a selective week in August 2018 with many different chaplains visiting with patients of all ages suffering from various illness. I suspected correctly that, by following the chaplains, I would find in myself, and in each patient visit, a more constant and heightened awareness of who the person in front of me was. I was right also in thinking the chaplains would demonstrate the enormous task of attending to where the patient is right at this moment while beginning to grasp the long arc of their life, and in fact demonstrate all those ineffable tasks that comprise the art of appreciating how our patient's illness is now bringing forth the stuff of their life for us to see, and to share in together.

During my week with the chaplains, what stuff I saw. I saw bitterness from a woman my age as she marked the yearlong anniversary of waiting in the hospital for a transplant. I saw the vulnerability of a dazed, naked 40 year old man about to be taken home by his family. I saw the inconceivable tragedy of a baby being hurt in ways that urges even someone nonreligious to ask where God is. I saw the armor of parents assuring us that all is well as they coddled their sick baby, having lost another a couple years before. I saw the tired eyes of a

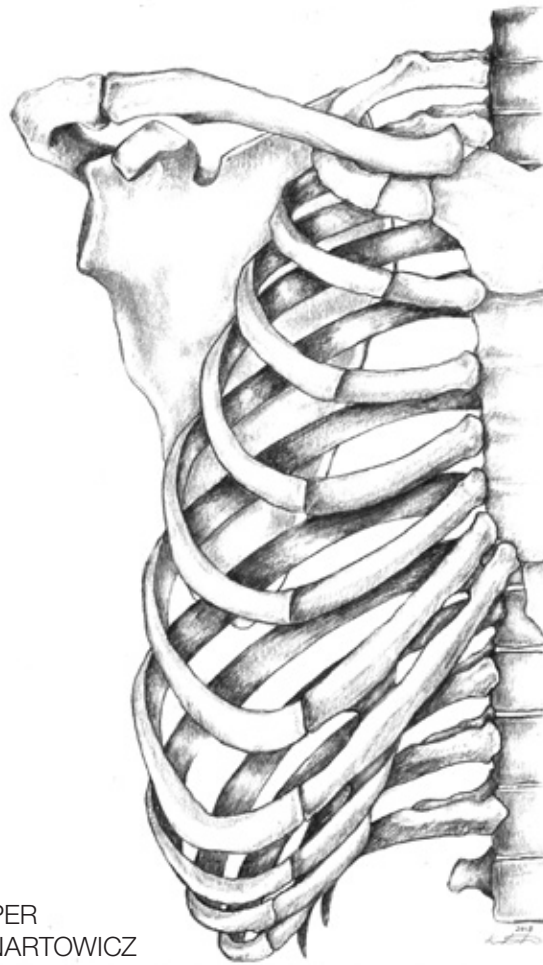
schizophrenic man my age, clearly battling demons only he could see. I saw the quiet sadness of an Alzheimer's patient, sitting upright but unable to look at me when I called her name. And on and on.

As all this stuff—rage, armor, frailty, horror, exhaustion, and on and on—rises up to the fill the space of the patient room, I joined each chaplain in his or her unique way of wading through this stuff to accomplish the same ends: to be a listening ear for festering woes, to set our patient's eyes forward towards hope, to help unearth an internal reserve of strength or faith, and most of all, to remind them of God's unwavering comfort and presence. In order to meet these ends, the chaplain steps through many tender territories. When sitting face to face with a suffering soul, there opens that wide expanse between deep understanding and misunderstanding, between vulnerability and guardedness. When approaching the patient's medical condition, the chaplain simultaneously evades and acknowledges the question of whether the pain and/or prognosis will improve. And as the chaplain steps through these and other territories sitting beyond my own view at this time, she steps with one foot in this world and the other in that World.

At the end of my week, I confirmed my suspicion that a chaplain's work in many ways is the very substance of my own inspiration for medicine. It occurs to me throughout the week, however, that my work as a doctor will look much different. I will enter the patient room wearing a different hat and singing a different tune. As I'm starting to see how much mental energy goes into remembering and analyzing the

basic facts of a patient's medical case, I wonder how much gas I'll have left to traverse this same tender territories. I remain anxious of my ability to perform this incredibly cerebral and emotional task of appreciating my patient's stuff, to wade through it to meet similar ends as the chaplains while also meeting my ends as the

doctor. I will end with my personal epiphany from this week: realizing that my anxiety masks a readiness, much deeper than I thought, to spend the rest of my life learning and trying to build a similar presence for my patients.



RIBCAGE

PENCIL ON PAPER
BY KARINA LENARTOWICZ



INTIMIDATION

BY KEKOA TAPARRA

THE CAVE

BY SAM ROULEAU

Needing pain still, I come
when I can, to the place
where we shared a now forgotten memory.

When we were young, the cave was buried,
until
the sadness began rising from its depths,
traveling its lingering passages consumed
me and
eventually, you too.

With each day, with each year,
the cave drilled me down, until
I was more shadow than man,
succumbing to starvation of spirit.

And, while you watched, I lost hope:
In you, me, us, our dream
forsaken.

At first, my apathy drove you mad, until
silence. Then, you too
knew the cave.

Together we languished in solemnity,
we dissolved into despair,
and you walked in
leaving me behind, then .

*July 14, 2018
Iowa City, IA*



LOST IN THE DESERT

BY RAMYA RALLABANDI

APPLYING FOR PERFECTION

BY GOHAR MANZAR

as they pick and choose the flawless among us
you begin to see any scuff or fault as grievous injury
rejecting the worth of learning how to swallow *failure*
“you’re not good enough for us,” they decree

and now in search of fortune, you reenvision
the very best version of yourself
you solve out the flaws that made you, like long division
and soundly leave them inside the doorstep

without a blemish, you step into the world now
empty of what made you but still smiling to yourself
it’s a while before the superficial glow abandons you
and you realize your (beauty) full past you must accept

because in regurgitating your accomplishments, you see
the empty calories you once inhaled now starve your soul
you fill a mold, unchallenged and faultless
when you rejected what makes you whole

do they not see, for one who hasn’t coped with disappointment
what will that unanticipated first time look like?
choked from an unfulfilled high ambition
the never fallen haven’t learned to break what becomes a steeper dive

when they—and we—pluck flowers from ugly cacti
we all forget that those lovely blossoms die
without the anchor of roots that may be rough
you’ll have never seen such beauty shaped into thrumming life



CACTI

BY REESE IMHOF

IN SEARCH OF A HOBBY

BY KEVIN MILLER

“Kevin, what *e/se* do you do with your time?”

The vague figure leans back and stares at me from across their desk. The phantasmal interview has gone well up until this point; the classic sticky situations were expertly maneuvered. Maybe my posture could have been slightly better. However, it would all be for nothing if I botch this pivotal question.

The interviewer digs in: “What are your hobbies?”

“Making peanut brittle,” I abruptly say.

“Excellent. I do that all the time. What type of peanuts do you use?”

“Um... I use... salted?”

And from this brief hesitation, it is clear to my imaginary interviewer that I am a fraud. For the record, I have no clue what goes into peanut brittle, besides sugar. I probably should have made it more than once.

My lack of hobbies goes way back. In fact, I don't think I've ever had one, outside of a brief stint collecting insects when I was six-years-old. In retrospect, it is probably good that this didn't persist. Would I be in a relationship now if I was still out there winning contests for “Best Moth”?

In my leisure time, I usually sit, maybe eat some pretzels, perhaps with hummus, maybe while watching television. Nothing out of the ordinary—*Game of Thrones*, old episodes of *The Office*. I occasionally look out of the window and contemplate if I

would enjoy bird-watching if there were less crows. I recently started trying sour beers more. I briefly lift weights a couple times a week. I rotate which coffee shop I go to depending on how I feel towards styrofoam. I watch a clip of Stephen Colbert on my phone most nights before bed. And that's pretty much it.

Why don't I have a hobby? I often loathe this fact. I usually blame it on others. Why did I stop collecting bugs? Or in its stead, couldn't my parents have forced stamp collecting upon me, and refused to let me to quit? Sure, it would have been painful. But, now at twenty-six, I am starting to see the benefits.

It seems that, to be likable, and thereby desired by a residency program, you need at least one hobby. It's essentially another box to check: research, evaluations, STEP scores, croquet. No one *actually* cares about your hobby, but it's vaguely reassuring to know that it's there, like your carbon monoxide alarm.

Hobbies seem to be a *de facto* good. An inoculation against burnout. It makes sense. You worry less about someone's mental state the more they are focused on yo-yoing or spear-fishing. Well, I'm not so sure about the latter.

The cut-off of general popularity for what can be classified as a hobby is nebulous, but it certainly exists. I would say if >7% of the population does the activity of interest, it is banal and prudent to not mention it. Yoga likely falls into this camp now. I think

rollerblading is still safe. Home beer-brewing is right on the cusp; mead-brewing, on the other hand, is [bleep] cool.

Hobbies are a luxury. If you have young kids, two jobs, or some other misfortune, you might not get to have one. And they definitely require cash. Along that line of reasoning, is it evil to ignore a homeless person on your walk to the taxidermy shop? Is that stuffed otter indispensable to your happiness?

I detest anyone claiming “movies” as their hobby, as if they watch them more deliberately *because* it is their hobby. However, those going to film festivals are absolved.

Running is a distinctly annoying hobby. So is photography. Owning a \$3,000 camera greatly increases the odds that you know absolutely nothing about how to use it.

A pet is something, but it’s not a hobby. That smiling parasite knows how to manipulate you, and prevent you from making homemade jam.

The culmination of my enmity is someone saying “travel” is their hobby.

Lately, if the “what do you do for fun?” question ever comes up in casual settings, I exclaim, “I have *no* hobbies!” It reliably kills the conversation. If it’s a close friend, they fumble around, trying to prove me wrong. “You like watching football... right?” If it’s my fiancé, she says, “Work is your hobby!”

Something has to change.

A past version of myself might have claimed cooking as a hobby. I did it a few times. And I love going into high-end kitchen stores. I have a meringue colored *Le Creuset* skillet on hold for when I drop down to five-figures of debt. However, once again, if pressed in an interview, and it was revealed that my caloric intake consists of roughly 50% scrambled eggs, I would be in a major bind.

I also might have claimed hiking as a hobby in the past. But can you really do that in Rochester, Minnesota? I guess you can walk around, but *hike*? I hiked in the Rocky Mountains few times and loved it, but that was like four years ago—too distant in time and space.

Playing a musical instrument is a *bona fide* hobby. If only it weren’t too late for me.

Is “reading” a hobby? I’m on the fence. I think it may be, but it can’t be *When Breath Becomes Air* (see 7% rule above). I’ve slogged through hearing that thing described so many times I can quote it word-for-word.

However, I can’t even claim reading as a hobby because I don’t read books. The only thing I do that resembles reading is when I go on Wikipedia for twenty-ish minutes to learn about a random topic—say, “The Dancing Plague of 1518.” But I can’t tell anyone this. It raises too many eyebrows.

What about reading the news? I spend a significant chunk of time on that every

day, but it would be odd to say I'm doing it for pleasure; compulsion seems more appropriate. I also do the *New York Times* mini-crossword every day, but it takes about 53 seconds on average—too little time to claim as a hobby.

I feel guilty about playing video games. People are out there *making soap*, and I am riding a horse around a simulated version of the Wild West and deliberating if my character's beard needs to be trimmed. Plus, if I mention that I do this while eating *Fruity Pebbles*, it just doesn't get the same reaction that I think I would get if I said that I enjoy smoking meats and playing badminton.

I loathe that I watch football, as does my fiancé, and probably our neighbors based on the Packers' performance this past season.

So where does this leave me for my residency application? I am happy, don't get me wrong. That sometimes comes as a surprise to me.

What if working *is* my hobby? I won't even go there. It's an undeniably hideous thought.

And this is why I find myself at the grocery store at 11:15 pm buying Spanish peanuts and a candy thermometer.



MEDICINE IN THE DESERT

DIGITAL SKETCH ON IPAD
BY TALA MUJAHED AND AUSTIN PEÑA

SELF-SERVICE • AND THE MATCH

BY GOHAR MANZAR

flattering ourselves, we find a compass
the good life *I want I need I crave*
an inhuman competitiveness of a dream
nurtures a formula by which we now behave

filling ourselves when this profession
should be all about giving to others
no chance to pause on the trail, even as we short circuit
they ask and we must deliver

while rattling accomplishments and ambitions
you try to stave off the emptiness inside that rots you
by selling a calling with irrelevant trophies
you bathe in the impurity of a process that defiles you

feeding greed and egos with awards now freshly dusted
form fields are madly filled with essays and lines
under the pretense of a healing venture
the soul worn for patients decays into a fashionable lie

and you finally pause as you wonder how
in the profession of helping others live
there is so little love and warmth in the process of
selecting through this harsh and discerning sieve

deciding "I like what I do" carries more weight than the heavy privilege
of holding someone's life in your feeble hands
abandon your awards, your honors, and your needy splendor
because you were called to listen, heal, touch, and understand

THE STORIES WE CARRY: ON HOPE

BY PATRICIA BAI

As future physicians, is it ever okay to give or take away hope, the very thing that sustains us when all else seems against us?

As I walked into the room where a couple sat side-by-side holding hands, I braced myself for what the doctor had told me about this case. We were about to see a sixty-year-old man who was receiving radiation follow-up after left-sided facial reconstruction from a surgery to remove a tumor. His wife's eyes were red—from tears or from the four hour drive over, I wasn't sure. His face was healing well, with only the droop of his left side as a reminder of the pain he had endured in the past two years, a droop that persisted even when he smiled. As he looked in the mirror, he seemed to be looking past himself to a time once lived before. Despite the doctor's insistence that this was an incredible recovery and that he was doing well, he turned and said, "I know I should be so grateful to be alive, but sometimes I just long to be normal again." Quietly, looking away, the doctor told him that this was in fact likely the best case he could have ever hoped for.

Days later, I met a patient who had a parotidectomy to remove a tumor—by all intents and purposes, he looked normal to the untrained eye. But upon further inquiry, he revealed he had lost all interest in eating food post-surgery. Nothing he enjoyed before tasted good. Eating was a horrible chore where he had to meticulously cut one piece into ten smaller pieces without the reward of what most of us experience as a taste bud explosion—in his words, it was like what being forced to swallow ten pills

each day was like to the average person. Because his insurance didn't approve proton beam radiation over photons, a concern that placed money over a patient's quality of life, his radiation therapy likely eroded away more of his taste function than was necessary. Despite this, he had to try giving food a shot, or else that small glimmer that he might taste again might fade away for good.

When the doctor showed me CT scans of a giant, metastatic mass in a young man's abdomen—one that had pushed and eroded away at the duodenum, my eyes widened. Renal cell carcinoma. A spotted kidney, so engorged by tiny masses it was almost unrecognizable. He had endured the pain of the tumors, the uncertainty of the diagnosis, the biological effects of immunotherapy, and the grueling nature of a surgery. Yet, months later, this defiant little tumor would not disappear, instead orchestrating a sinister invasion into the liver and the pancreas. Walking into the room, I expected to see a pale man who had put up a good fight but was tired of getting back up after repeated kicks to the floor. Instead, I saw a man full of hope, happiness, and witty remarks. His prognosis? At best 3-5 years to live, despite all of the efforts to cure him. His last hope would be the *abscopal effect*, an amazing phenomenon, so rare that most consider it to be a miracle. Would it be right to put this man through even more treatments, with all the side effects of radiation, for the miracle that he might be one of the lucky ones who defies the statistic?

These were stories of resilience, hope, success and loss. These were the stories of the pain of what cancer gives and swiftly takes away. These were the stories physicians held close to their hearts—the ones where they were forced to ride the fragile line between hope-giver and hope-killer. These were the stories of medicine exposed—the stories that left medicine's most vulnerable limits wide open for all to see, showing how little we know but also how much there has yet to be discovered. Decades ago, chemotherapy was something dreams were made of, yet patients and doctors forged on, injecting toxic chemicals through countless bodies in the hope for success. Today, we are the generation that reaps the rewards of more success with treatments, built off years of trial and study. Years down the line, would we make something that could elicit a miraculous abscopal effect with a more surefire certainty, a reality in which the young man with renal cell carcinoma may live?

In the face of great ambiguity and promising but not-yet-proven tools, what is a physician's role in these storylines? Do we hold steadfast to that glimmer of hope and encourage the patient to keep fighting while we stand and watch what we cannot fully comprehend from the safety of the sidelines? Or, is it fair for us to take that hope away, backed by the swift pronouncement of a statistic and a regretful gaze that apologizes for the fact that we can't do more?

It's these stories that I will remember, these patients who I will continue to think about, wherever I go in medicine. When I think about the people I met this past week, I think back to a poster that was hung in one of the physician's offices. Smack in the center of dozens of powerful quotes authored by survivors of cancer, stood a phrase to sum it all up: There's no place like hope.

REFLECTION

OIL PAINTING ON CANVAS BY JENNY YOON



NATURE'S PALETTE ON A CANVAS

BY ANJALI PANICKER

GIMME SHELTER

BY ALYSSA BROWN

"It's just a shot away, it's just a shot away," blared the music in the OR. There was a flurry of hands and spurts of blood. But I should start at the beginning of the story. I hate to say that there is something exciting about getting called in to the hospital in the middle of the night. Logically, I know that means something bad is happening to someone else, but it makes my heart beat a little faster and my adrenaline rush to know that I can potentially help. I had been following a surgeon at a local hospital all Saturday. We didn't have any cases so we both went home to rest at 3 p.m. We agreed that she would call me if anything came in. She always rolled her eyes when I asked her to do this because she thought I should probably just get some sleep and enjoy my weekend. I got a text from her at 8 p.m., and it said, "Seems like a quiet night, so unless something changes, let's meet at 8 a.m. to round." She had uttered a jinx by saying it was a quiet night, but I decided to go to bed early and turn my phone up loud just in case.

At 1 a.m., I sat straight up in bed—my phone was playing its loud jangling tone. I didn't recognize the number, but I went ahead and picked it up after rubbing some of the crust out of my eyes. "We have a case. You don't have to come in, but if you do, come through the ED, and I'll prop the door open so you can get up to the lounge. Be here ASAP if you're coming: this case needs to go fast." I quickly woke out of the fog. Before I even responded, I was pulling

on a pair of scrubs. I grabbed a pair of diet colas and a granola bar before half-jogging out the door.

It was pouring rain and there was a chance of the roads flooding that night. I didn't think about it as I tore down the road probably a little too fast. My mind kept rifling through the possibilities of the case. She hadn't revealed any details, but I knew that it probably wasn't an appendix, which is usually not a middle of the night emergency operation. It was definitely not trauma because the small hospital was not a trauma center. I thought might be a gallbladder, but that can usually wait until the morning as well. I whipped my car into the ED parking lot. I walked into the front door of the ED and past all of the security and staff, and I shimmied through a door into the staff elevator. I slid through the propped open door to get into the PACU and down the hall to the lounge. I took a deep breath before walking into the lounge.

I found the surgeon curled in a chair, covered in a blanket fresh from the warmer. She was reviewing the patient's chart. I handed her the diet cola that she didn't remember she needed. She was scrolling back and forth through a CT scan. She told me to throw my stuff in her locker and look at the CT. I wadded up my white coat and crammed it into the top of the locker and hurried back out. Without turning, she said, "What do you think of this CT?" My brain was still foggy with sleep, so I wasn't sure

what I was looking at, but it looked like there was a lot of bright contrast sitting in the peritoneum, which I knew wasn't good. She began to fill in the details to add color to the black and white CT scan. The patient was an elderly female who fainted that afternoon. By that night, a CT scan had been done in the Emergency Department, and blood was visible in the abdomen. Radiology reported a possible splenic artery aneurysm that had ruptured. That's when the surgeon had gotten called. It was her turn to try to make this patient better.

She looked at me, and said, "We are going to get in there, get out, and pray that we can patch it up enough for her to make it through the night." As if on cue, the intercom screeched to life and said, "We're ready for you." The surgery floor always seems to be so full of life during the day, but at night, it almost seems haunted. There are empty beds, empty carts, and you can only hear your own breath—punctuated perhaps by the hum of a rogue machine. We walked down the hall towards the OR. I had seen her work her magic before, but not on a case like this. She was usually bubbly and talkative as we strolled to the OR, but tonight she was quiet. She knew that the odds were not in her favor. She was the last chance to save this woman.

The patient was moved into the room seamlessly. The gloves and gowns seemed to appear on the surgeon, like a quick costume change, and everything seemed to quicken. The surgeon strode over to the table and stepped up onto two step stools. As I stepped up to the table, my mind began to wander. I thought to myself that this could be my grandmother, or someone else's.

It is easier once the face is covered to not think of these more emotional details of the patient. It is easier to keep distance when they are just a case or just an anonymous patient on the table.

Before I got too deep into these thoughts, the surgeon called for a #10 blade and traced it down the middle of the patient's abdomen. The suction was shoved into the fresh wound as we went. Blood began to run down the sides of the drape. When we could finally see the abdomen open before us, there was dark red, congealing blood pooling throughout. Everything and everyone was at the ready. Hands seemed to just know exactly where to go. Nothing was fumbled. There was a steadiness to the surgeon's voice, with urgency but without harshness. The aneurysm of the vessel was hiding within the yellow clumps of fat, and it stubbornly did not want to give away its position. The surgeon began to pack the abdomen with laparotomy pads. She was shoving them down into the crevices and cracks of the abdomen to soak up any extra bleeding. We would leave them there. She dug and dug for the stubborn aneurysm, and finally decided coming back was a better option. The patient would be closed another day, but for today, she would leave the operating room with a new vacuum-sealed abdomen. The surgeon asked for lab values. The anesthesiologist recited them to her, and I could see her begin to frown even from under the mask. They did not sound good, but we had done all we could do for the night. Hopefully she would make it through the night.

We stepped out of the OR and peeled off our blood-covered gowns. I looked down

and realized that blood covered my clogs because, as usual, I had forgotten to put on shoe covers. It was 3am. We agreed to meet back at 7am to round and check on the patient. I was exhausted. I was also worried that the patient would not make it to the morning. She was in a precarious state, and I was not sure if we could pull her back from that cliff edge. I came home and kicked off my clogs and fell asleep in my scrubs.

The sleep came quickly, but my alarm came quicker. I was already tense about seeing the patient. I wandered through to the OR lounge where the surgeon was still staring at the CT from the night before and some new lab results. I could tell from her expression that it was not looking good. She didn't turn to me when she said, "We're taking her back to the OR." She stood and quickly paced down the hall; I didn't ask where we were going because I knew this was not the time for questions.

Eventually, we ended up in the family room of the ICU. The family members were standing there clutching coffee and speaking in hushed whispers. They quieted more when we appeared. The surgeon said, "We are happy she made it through the night, but she is going to need another operation this morning to keep her alive. I cannot promise anything. Her chances are not good, but we will do the best we can." Somehow, the family did not look surprised, or maybe they were still in a state of shock from the night before and hadn't yet absorbed this new information.

The surgeon began to assemble the team. She listed them like a batting order—her surgical partner, a hematologist, a critical

care specialist, anesthesiologist, and a first assistant. The patient was full of tubes, hoses, and wires, so it was difficult to move her to the OR floor. We crammed into the elevator and hoped that nothing bad would happen on the way there. Once in the OR, there was a flurry of activity. Things were going to go quickly again. The critical nature was weighing heavily in the room, but everyone there knew that it was a Hail Mary, and this team was the final chance the woman had to live.

There is nothing quite like a surgeon in a critical case. There is nothing wasted in their movements or words. The team worked like a machine. It was like everyone knew what the other would do before they did it. The wound vacuum came off, the staples were removed, and the abdomen was opened again. It seemed as though time stood still. It was only 8 a.m.

"The flood is threat'ning, My very life today, Gimme, gimme shelter, Or I'm gonna fade away," wailed Mick Jagger. It sounded like someone had turned up the volume. I could see the surgeon clipping vessels and saving this woman on the table as the song seemed to get louder. The two surgeons worked together to fix this. Music makes the OR feel special, magical. There are sometimes indescribable moments in the OR that don't seem quite real—a suspended disbelief, where miracles seem to appear out of the ether. I thought for a second, there is no way that she isn't going to make it off the table, not with the feeling that was filling the room. The room collectively sighed as the surgeons began to navigate the patient back from the brink. The bleeding was controlled, and everything was working perfectly. Before I knew it, the

case was over. The bubble of magic had burst, and the patient had survived another hour. As we all began to strip off our masks and gowns, someone pointed out that it was an all female team. There were leaders, but it was a group effort rather than a single person who saved this patient's life, at least for a while longer.

After the surgery, we continued to make rounds, but kept coming back to the patient. We kept wandering by her room to talk to her family and update ourselves on how she was doing. By the afternoon, she remained stable. She was not awake or out of the woods, but her lab values and vital signs were improving. This was my last day with the surgeon because I had to go back to my normal rotation on Monday. I finally went home.

I woke up Monday and started my first day of a pediatrics rotation. I kept thinking about the weekend patient, though, and I was worried. How was she doing? I was afraid to ask. I was afraid to hear that she had passed. I thought of texting the surgeon, but I did not want to bother her after an arduous weekend.

That night, I heard my phone buzz, but I didn't think much of it. I finally checked, and it was from the surgeon. My heart dropped into my toes. I was afraid to open it. I knew before reading it what it would most likely say. It said, "She went into pulseless electrical activity this afternoon, and we never got her back." I responded that I was sorry, and I knew she had done all she possibly could. She had given it her all. She responded, "Thank you." For one of my first times during third year, I felt like the physician had the same feelings about the patient's death as I did. We were both hurting. I could tell she was grappling with what she could have done differently. She didn't blame the other team members. She assumed full responsibility.

Still, when I hear the song "*Gimme Shelter*" on the radio, I think of that patient, that night, and the magic of the operating room. Some nights, all that is left is a Hail Mary for the patient, but sometimes, the miracles do happen, and sometimes the true miracle is keeping them alive long enough to say goodbye.

ROSE-TINTED GLASSES

DRY-ERASE MARKER ON WHITEBOARD
BY BENJAMIN NELSON



SHE SHOWS ME LOVE

BY TORI RICCELLI

A splash of soft snow between two eager
eyes
looking up long before I had lagged in
Lying on a worn bed,
well-loved
well-used
A pink patrick star rested under her warm
paw

Thumping, thumping out her language of
love

It makes me wonder and I question why,
why don't you judge me like the rest of the
world?



MOM IN ROME CIRCA 1985

PENCIL ON PAPER
BY MAX STAEBLER

DEAR BROTHER

BY MYLAN BLOMQUIST

For anyone with experience with adoption, and for anyone who is learning their definition of family.

Dear Brother,

How can I catch you up on a few lifetimes? Not just my own, but also the ones that came before me. The lifetimes that made both of us; the lifetimes I carried with me, wondering if I would ever meet you to share them. All the lifetimes I invented for you before we met. You have taken on a million different forms and worn a million different masks in these years that we lived under the same golden sun. Now, you have a face, a house, a family. Our meeting was not the last puzzle piece placed to complete the picture, it was the making and cutting of the puzzle itself. Although the puzzle exists and forms an image, it is cut, mixed together with the shake of a sturdy cardboard box. It is no easy task to put these pieces together, nor is the story even close to over. I know some of the images and have some of the pieces; you hold onto others. Most of these pictures do not come from a camera; if only I was lucky enough to be able to capture every moment, keeping them forever trapped in such a concrete medium. Unfortunately, the passage of memories, trauma, and love from generation to generation is a more ephemeral process, one that can only be felt. The best way to get you caught up is to tell you about the puzzle pieces I have and the images that they make.

Peggy: a vision in turquoise. Our grandmother. Like most fashionable women in 1960s Arizona, she sported geometric

statement jewelry featuring stones forged not in fire, but the slow melodrama of erosion. Whether she related to the melodrama or to the erosion more, I will never know. What I do know, is that each day she went about her life as a homemaker and novelist, clanging out the details of her lavish imagination on her typewriter, she carried with her a deep well of pain. The water of this well never dried up. In those days, it was hard to know what sun held power enough to evaporate the water in her well. Without finding that sun, she turned to poison to wear the pain down. I'd like to think she was not aiming for scorched earth, but instead trying strategically to lure the pain out into the daylight, where she could poison it little by little. On a fateful October day, the desert sky unfalteringly, impossibly blue, she tragically became the collateral damage of this poison. Her life was far from complete. I wanted to be there, to grab her hand, and ask her where she was going after getting off the phone with the publisher. Peggy never knew that you and I would come to exist.

The poison seeped from past generations down onto Peggy, and also down onto her son. In a few decades, only Peggy's daughter would remain, who became our mother. Our mother was and still is resistant to the poison. It beads up and falls off of her, unable to withstand her steadfast determination to nurture others even at a great cost to herself. Lots of people who love their parents speak well of them, and

even more talk about their generosity. It takes generosity and patience to bring other humans into the world in the first place—but I'd like to think the actions of our mother are overflowing from the unlive d life of Peggy before her, and the unlive d lives of who knows before Peggy. After you and her parted ways, her struggles were not over. Despite this, she brought so much light to the shabby little neighborhood I grew up in. Some of our neighbors lived in cars, lived in backyard sheds, lived with their abusers. She saw and continues to see these people as family, and did whatever she could do for them. I watched her call 911, I watched her console our neighbor's daughters after their mother died, I watched her help other neighbors with their overwhelming amounts of paperwork for all kinds of assistance, I watched her help my dad, her husband, through episodes of serious illness. She has always been a fixer and aptly found her calling as an editor as my sister and I grew up and left home. In her tradition of doing enough living for all her family members that left her behind, her life as an editor gradually transformed into the life of a writer; and then a biographer of Holocaust survivors. I think she doubts her abilities as a curator and good steward of these important stories, but I think she is more qualified than anyone else. She has been a part of others' stories for years.

What puzzle pieces tell my story? Like our mom, I think I am better at telling you about everyone else. This chapter of my life, the one in which I met you, started on the summits of Zion's mountains and the sides of its cliffs; I felt washed smooth as a river

rock by the cold water of this holy land. After that, I left the southwest to start medical school. I left the Valley of the Sun behind, the Valley we shared for so long without realizing. I bet you know the way that this city can nurture you and burn you up at the same time, and I was no exception. Zion forgave me, and apologized for the sweltering Phoenix nights that robbed me or broke my heart. It's not personal in the southwest, where so many people melt together and live their confused lives in this young city, free of any binding history but also lost without it; it just is.

I visited Peggy's grave on the 4th of July last year. The heat distorted the black asphalt and made the air shimmer. I was the only one around, aside from a few swans gliding on the partially evaporated concrete lake. I imagined our mom standing next to this grave when it was fresh. She would have taken you there if she could have, I know it. Residing in the grave next to Peggy's, a kind and thoroughly loved gentleman, I imagine, sported a tiny American flag and a cold beer in a bucket of ice stuck in the grass next to his gravesite. "I'm sorry," I whispered to her.

Maybe we can go together next time.

Love always,
Your sister

SUBMERGED • HAWAII

BY KEKOA TAPARRA



The ocean has played a big part of my life. It is one of the places I truly feel at home and at peace.

BRINE AND ICE

BY SAM ROULEAU

Shuffled steps behind five corgis and two proud owners,
behind me a recalcitrant son and sperry clad dad
who hates me for jumping off the path and bounding over the rocks before the last bend.

The cascading clouds break for winter sun atop the ocean
whose seaweed mats crest with each break. I want to dive in and feel
the cold stop my heart

so I may rise anew in a coat of kelp.

We walk to Mike's crab shack and eat twenty-dollar lobster rolls.

January 10, 2019

Wells, ME

ICE CASTLES

BY STEPHANIE ANGUIANO-ZARATE



BOUNCE DAY • 2018

BY BRANDON GHISLAIN

Last Sunday, I survived a Zombie Apocalypse.

It was certainly a cordial apocalypse, to be sure—with well-controlled chaos and prompt punctuality, which is what I tend to like in apocalypses, generally.

The setting is an old crumbling castle on a hill. A crashed bus. A zombie plague. A horde of terrified, wounded refugees. Half-a-horde of terrified, volunteer, emergency-response-pretenders. The day is Bounce Day: disaster simulation. And we had been prepping for roughly a month leading up to it. Tourniquet training. Chest tubes. Triage.

That last one turned out to be crucial for me, since I was sorted onto Jump Team (read: Triage Team) for our first simulation. We were the first folk on the scene when the impatient white truck slammed into the backside of the slow, dusty school bus, so we rescued four survivors from that truck on the quick. Two walking wounded. Both green. Good to go. One gasping, but still drawing air and well-oriented. He knew his name. Knew what had happened. Also green, good to go. The last one had a windshield shard sticking in her eye. Terrified. Frantic. Plainly in pain, but not badly bleeding. Not presently dying. Gruesome, but still not a red. Yellow then.

We tended to others, moving onto the bus—making sure everyone aboard was okay. They weren't, but that's not this story. Disasters are hard. There is so much going on. You have to pick where your attention

goes. Partition it. Fast forward here a bit. Hold our focus. After clearing the bus, we mounted a search and rescue operation to recover two more survivors from the woods.

By the time we got back, Jump Team had mostly finished. All the survivors had been triaged already and had been funneled into either the refugee camp, the quarantine, or the hospital. And by this point, with our initial excitement in waning, I found myself feeling somewhat useless. I asked where I could help and was pointed toward the helicopter extraction site to tend to whoever needed tending.

And that's where I found her.

My windshield-eyed survivor. The girl from the truck. Waiting there, sitting with her back against the stone wall. She was wailing, weeping, and banging the back of her head against the bricks. She'd been driving the truck that had struck the bus. She'd been irritated and had wanted to pass. But she'd missed. The crash was her fault. The disaster was her fault. She spoke not a word about the windshield in her eye. She only kept wailing her repentance.

I asked for her name, what she did for a living. She worked at a bar, she choked between sobs. And on Saturdays she ran karaoke. What was her favorite song? She couldn't think, so she asked for mine. Journey: *Don't Stop Believing*. I asked her to sing it; everyone knows it. She couldn't at first, so I started us off. "*Just a small-town girl—living in a lonely world*". That was

enough. She knew it from there. Still half-sobbing, she sang: "*Took the midnight train going anywhere*".

We finished the verse. We got to the chorus. Other folk around us joined in. Good. *Perfect*. That was what we were going for here. Community. Connection. Being *part of* something, not the cause of something. Being in this together—here with what's happening—not trapped in what was already done.

As medical students, one of our pre-load training sessions for Bounce Day had taught us Psychological First Aid, and I really took that lesson to heart. There are five principle pieces to psych first aid:

1. making people feel safe
2. calming them
3. providing a sense of self-efficacy
4. promoting connectedness
5. offering hope

"Don't stop believing. Hold on to that feeling. Strangers. People ... "

Music is amazing. One song hits three out of five. She was laughing. She was with us. For a moment she forgot. For a moment, she wasn't the cause of a zombie apocalypse with a chunk of glass stuck in her eye—she was a young woman singing an epic, great rock song with half a dozen folks who were all in this together.

Her story darkens again a bit later, but I think we can end with the light for today. A moment of light in the midst of her darkness. She needed it more than tourniquets or chest tubes. And probably so did I, being honest. A tiny light. A candle at best.

But we should never underestimate the comfort in candles.

We should never underestimate the light.



STARBURST

ACRYLIC PAINTING ON CANVAS
BY TALA MUJAHED

TRUSTING OUR DOUBT

BY ADIP BHAGRAV

Trust lies at the cornerstone of medicine and is an essential requisite for competency as physicians and teachers. Fostering trust promotes effective healthcare delivery and an atmosphere that is more conducive for learning. When we trust a healthcare provider, we believe that they will act in our best interest by competently and thoughtfully performing their roles. This trust is typically founded on the merit, reliability, and track-record of the individual. However, trustworthiness and competence are increasingly conflated with certainty and overconfidence, especially in academic medicine.

In interactions between instructors and students, the misconception that an overconfident trainee is trustworthy and competent undermines the learning environment. Reinforcing clinical decisions made with false certainty sets a dangerous precedent about expectations of trainees. When trainees feel compelled to appear certain of their knowledge and decisions in order to be looked upon favorably by supervising instructors, there is a potential gap of knowledge that remains unfilled or worse, leads to poor clinical decisions. Additionally, research suggests that those

who claim to be expert or have high certainty in a particular area are prone to thinking that they have more knowledge than they actually do.¹ It is also theorized that those trainees that may actually be incompetent are in fact more certain and overconfident since they may not possess the knowledge to recognize their own limitations.² Taken together, if the *appearance* of competence and trustworthiness is reinforced rather than true competency, then a pattern of knowledge gaps and a reluctance to seek constructive input emerges in trainees causing a failure to achieve learning outcomes.

Instead, instructors should encourage healthy skepticism and doubt in trainees. As William Osler describes, “medicine is a science of uncertainty and an art of probability.” This should be at the heart of medical training. Encouraging trainees to feel comfortable to grapple with uncertainty of their knowledge would create an environment that promotes the achievement of true competence and trustworthiness. By shifting the culture to trusting our doubts, there is potential to enhance the trust in one another which is crucial in robust and high-pressure healthcare settings.

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INTIMACY: REFLECTIONS ON HUMAN ANATOMY AND THE DOCTOR-PATIENT RELATIONSHIP

BY JON SUSSMAN

I'll begin with a tangent, because I am a bit of a tangents guy. On December 13th, 1991, Dr. Mark Siegler delivered the Rouse Lecture to the Mayo Center for Humanities in Medicine with the title: "Falling Off the Pedestal: What is happening to the traditional Doctor-Patient relationship." He sought to answer two questions central to the field of medicine and its sacred dependence on the public trust: first, at a time in which medical science is more advanced than any prior generation could have dreamed, why has the tension and distrust between doctors and their patients increased? Second, will future advances in medicine wrestle autonomy and decision-making power away from patients and physicians and into the hands of other parties?

Dr. Siegler, who I spent four years with during my undergraduate years at the University of Chicago, was decrying a landscape in 1991 in which 26% of Americans respected their physicians less than they had 10 years prior and 50% reported believing that their doctors care less about their patients than they did in the past. Today we deal with a healthcare system in which appointment times are shrinking and according to a New York Times article from January 2018 only 34% of people express great confidence in medical leaders. In 1966, one year before Dr. Siegler received his medical degree, this percentage of Americans expressing great confidence in their medical leaders was 75%. The problem is only growing and a new generation of medical

leaders, educated here at Mayo Clinic and elsewhere, must be ready to take on the great challenge that this perceptual shift presents to the legitimacy of medical practice.

Now that I am off of my tangent, the topic at hand today is the deep gratitude that each of us medical students feel towards our first patient: the individual who donated their body to our anatomy program. These individuals were so selflessly kind that they entrusted their bodies to the betterment of our education. It is difficult to summarize the true importance of human anatomy in a physician-in-training's life, because the experience affords so much more than anatomical understanding. It is our first encounter with our first patient.

Our first patient. The essential uniqueness of forming your own relationship with an individual that wanted you to understand them in every way you could. To become familiar with the common humanity in all of us with them as example. To witness, firsthand, the previously unfathomable depth of biological similarity that unites us all in shared experience and to sit in awe of the complexity of our shared physiology.

The willingness to allow us, the students, to explore this intimate and personal connection for the first time is one of the more special gifts I could imagine receiving. Donation is an affirmation of our trustworthiness as medical professionals and sets our moral compass towards maintaining the trust that was afforded to us

by this leap of faith in our abilities. If we are to live up to our aspiration of being effective partners in these tortuous journeys, then this first experience is of the utmost necessity to our personal and moral development.

The greatest gift that medicine has received across its wide and often troubled history is the public trust. This is a trust that manifests in every doctor-patient relationship. It is at once susceptible to great abuse and also miraculous goodness that has no similar parallel. This potential to intervene with the consequence of both detriment and benefit hinges on the special intimacy that defines this unique form of trust. There truly are no individuals, aside from one's family, that are afforded a similar level of trust. This is what empowers the medical community and obligates it towards the great responsibilities that are entailed in this trust.

Medicine's central goal is to address the many needs that result from an imperfect world through a relentless focus on

the accumulation of knowledge and its compassionate use to ameliorate the suffering of others. It can only manifest its benefit through the medium of this special trust.

The problems that Dr. Siegler decried in 1991 are well and alive throughout medicine today. A stated goal of Mayo Clinic and its Alix School of Medicine is to educate future physician leaders to address these problems in novel and creative ways. Through the donation that your family or friends have provided to us and the trust that becoming our first patient entails, the true moral gravity of this aspiration has been revealed to us. The intimacy and the power of the doctor-patient relationship are now manifest in our everyday experience. Thank you all, and thank you to our first patients, those who taught us more than any classroom experience ever could.

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THANK YOU, MAYO PEOPLE

BY DEEYAR ITAYEM

I get asked often—by friends, by family, by medical student colleagues at other institutions, by my patients, and by interview applicants on tours—“What is it that makes Mayo so great?” While I appreciate Mayo Clinic and the person it has shaped me into, this question is always a difficult one. How can I encapsulate all that defines “the Mayo sauce” without losing my audience in rehashing all the ingredients? Like many at Mayo, I resort to the answer “*the Mayo people*.” I feel that these simple words simultaneously capture the essence of Mayo in an understated Minnesota fashion. However, it wasn't until recently that I consciously realized that my answer holds a much deeper meaning.

Mayo is more than its humble, kind people who are sparking innovation in every field. Mayo is more than my brilliant and down-to-earth classmates who will undoubtedly change the landscape of medicine. Mayo is more than a physician's ability to pick up the phone and call a world expert colleague to ask an opinion on a whim. Mayo is more than our fabulous nurses—without whom we would frankly have no hospital. Mayo is more than the people who ensure patient safety, staff safety, and student safety. Mayo is more than the behind-the-scenes secretaries, janitors, researchers, administration, and others who know their work is valued. Mayo is more than the sisters who root us in their God-ordained mission. Mayo is more than our ability to share our appreciation of fellow staff in small daily ways. Mayo is more than the donors who understand that the mission comes

at a monetary cost. Mayo is more than the beautiful building infrastructure that instantly signals to patients and visitors that we are grounded in patient values (pun intended). Mayo is more than the magnificent Dan Abraham Healthy Living Center and the immeasurable physical/mental wellness that it has provided us at a subsidized cost. Mayo is more than the numerous committees who have brainstormed to make Mayo an even better place. Mayo is more than its forward-thinking leadership that remind us why we wake up every day... or in the middle of the night...or why we actually don't sleep some nights, against our own medical advice. Mayo is more than the surgeons who spend dozens of hours or even a whole day operating (my advice: compression stockings have been a game changer as I try to emulate their almost superhuman capabilities). Mayo is more than the creative social workers who traverse difficult circumstances artfully. Mayo is more than our internists, critical care specialists, and emergency physicians who skillfully navigate every organ system. Mayo is more than its psychiatrists, neurologists, and neurosurgeons who are quickly gaining ground in understanding the complex brain. Mayo is more than its physiatrists and physical therapists who know that rehabilitation is powerful. Mayo is more than the child advocates and pediatricians who know that our children are the future. Mayo is more than its allied health providers who strengthen our practice. Mayo is more than the interpreters who ensure that we tackle patient barriers one language at a time. Mayo is more than the cultural pioneers who

understand that diversity is being invited to the party but inclusion is being asked to dance. Mayo is more than the courageous patients who travel across states, countries, continents to get here.

Mayo is all of these things and *more* because of the collective *values* and *identity*

we share in our supportive community. But until I can concisely shorten this previous paragraph into a sentence, I'll stick with "the Mayo people" as my answer.

THE OUD

BY AMAL CHEEMA

The Maghreb moon reprieved the reckless rays of Apollo.
Among a thousand bounding shadows, I stood alone,
Immured by rose-stained air that enraptured an old sovereignty,
That had once surged with echoes of ancestral, sacred sounds,
Which had faded within the annals of myth and record,
So brazenly erased in the eroding of warring tribes.
I waded amid the waning desert sea of a city
That did not burn heedlessly, to hide astronomized antiquity
Behind the tapestry of delirious day and silent smog.
There, I saw you waiting by the sun-shorn, wind-worn walls,

Yourself, weathered by distress and redress of which Atlas only knew,
And too excluded from the waves of laughter of acquainted rendezvous.

The familiar face of an unfamiliar wanderer, how strangely you struck me.
Your gaze deepened at the chaotic song of the stone-strung fortress,
Swept by and in the catenated melodies of lands unknown,
Before you too slipped away amid the shadows,
Seized by the swells of sanded faces —
And with you, an opaqueness that no longer obscured me.

January 15, 2019

The Oud is a memory from when I was solo traveling and attending a festival on sacred sounds in Fes, Morocco. I found myself lost in a crowd, amid listless wonder and engulfing stories, when I noticed someone, who I felt was similar to me yet seemed more assured of himself. I'm not sure what drew my attention to theirs in a sea of anonymous faces, but I thought then of the beauty of human interaction. Yesterday's stranger could be tomorrow's friend; and, I didn't feel so alone anymore



MAGHREB: CAMBODIA

BY RAMYA RALLABANDI



IN THE LIMELIGHT

JACKSONVILLE, FL
BY ANJALI PANICKER



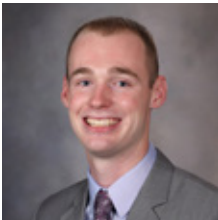
Gohar Manzar is a fourth year medical student who will pursue residency training in Radiation Oncology at MD Anderson Cancer

Center, with a Transitional Year at Memorial Sloan Kettering Cancer Center. She grew up in Iowa and went to college in Texas before getting her Ph.D. in Biomedical Engineering from the University of Iowa. She stays busy finding the best taro bubble tea, making elaborate DIY gifts, sewing, writing poetry, crying over her own stories, engaging in graphic design, and most recently, playing the ukulele badly.



Noelle Driver is a third year medical-master's student from New Jersey. She marvels at the wonders of mountains, human

compassion, fractal geometry, and Microsoft Excel.



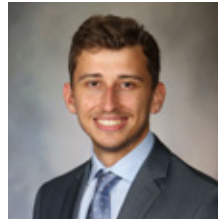
Sam Rouleau is a second year medical student. He is enthused by sloths and dessert. On a Friday night, you can find him on the yoga

mat and on the climbing wall.



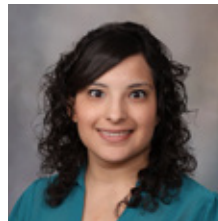
Charlene Gaw is a rising fourth year medical student from Northern California, who loves living in Minnesota for the Mayo community and the

beautiful seasons. She enjoys merging her passions for politics, healthcare, and yoga.



Max Staebler is a third year medical student from Florida planning to specialize in Urology. When he isn't re-watching The Crown or Star

Trek: Voyager for the ninth time, he enjoys planning his next target trip and having loud and drawn-out conversations right by that squat rack you need. In his free time he enjoys cultivating a sophisticated assortment of dank memes, slow cooking meats and baking multiple loaves of bread a night. Look for him soon at a NYC bodega near you.



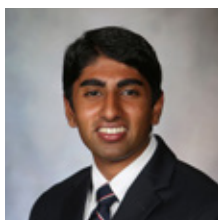
Stephanie Anguiano-Zarate is a native from the Juarez, Mexico-El Paso, Texas border communities. She is a fifth year Clinical

and Translational Science Ph.D. candidate. In her spare time, she enjoys listening to podcasts, hiking, photography, crafting, and maintaining her pet hedgehog's Instagram account (@hedgehog_chronicles).



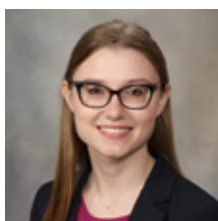
Patricia Bai is a first year medical student from Baltimore, MD with a passion for pediatrics and design thinking. In her free time, she

loves exploring farmers markets and cafes, rock climbing, and competitive games of bananagrams.



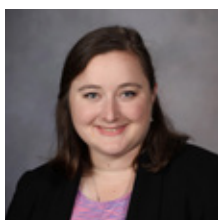
Adip Bhargav is from Ellicott City, MD but has bounced around from India to Wisconsin, Tallahassee and Atlanta. He is an

avid tennis and ping pong player and loves to write and draw. Adip is fascinated by the intricacies of the brain and the disease process of cancer. He hopes to find the intersection of these interests and incorporate them in his future medical practice.



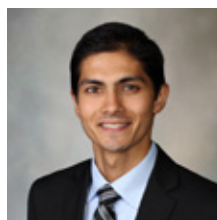
Mylan Blomquist is a first year M.D./Ph.D. student. She was born and raised in Arizona and studied biology at Arizona State University. She aspires

to be an oncologist and loves lab science, reading novels, climbing big rocks, and her cat, Theo.



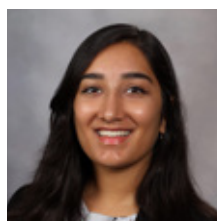
Alyssa Brown in a previous life, a.k.a. this summer, finished her third year of medical school at the University of Louisville School of

Medicine, and then was crazy enough to move to Minnesota to be a Ph.D. student in the Biomedical Engineering and Physiology program here at Mayo. She does not enjoy this much snow because it is getting ridiculous, but she does enjoy exploring new breweries, stress baking sweets, and following pediatric surgeons. She is grateful that The Tempest exists to provide an outlet for things experienced and seen in medical school, because it can be hard to cope sometimes with patient tragedies.



Kris Chatterjee is a second year medical student from the Chicagoland area interested in psychiatry. He likes cheap wine, The

Jimmy Kimmel Show, and calls his mom every day.



Amal Cheema is passionate about global health, policy, and ethics. She currently is a research assistant (GREP) in the Biomedical Ethics

Research Program. Before moving from Massachusetts to Minnesota for the warmer weather, she studied the intersection of religion, culture, and organ donation in Southeast Asia and Europe.



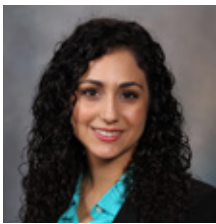
Brandon Ghislain is first year medical student who doesn't have nearly enough time these days to write. He is a former Shakespearean and

musical theater actor who has—for reasons obscure even to him—a degree in physics and math. His sci-fi and satire shorts have been published in Analog, Specklit, and The Satirist (under the pseudonym C.S. Lane). He is enormously interested in nervous systems.



Shemonti Hasan is a fourth year medical student from Chandler, Arizona. She recently matched into neurology at Mayo Clinic Arizona.

When she is not thinking about the brain, she enjoys dancing, drawing, reading comic books, watching anime, and, listening to music from Studio Ghibli. She is grateful to Mayo Clinic Alix School of Medicine for providing the foundations to become a caring and competent physician. She looks forward to further growing those skills during residency.



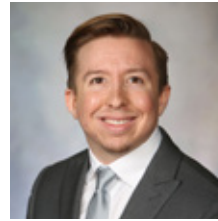
Tina Marie Hendricks is a third year medical student pursuing a career in ophthalmology. Outside of her medical education,

she is passionate about innovation, entrepreneurship, and relationship. In her spare time, she enjoys creative writing, backpacking with her husband, and making up recipes in her kitchen.



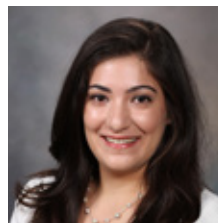
Oluwatomilona "Tomi" Ifelayo is originally from Nigeria, but currently lives in Houston, TX. She graduated with her B.S. in Biochemistry

from Baylor University. Tomi is a second year medical student who enjoys reading and writing and is still navigating what it means to be a student doctor.



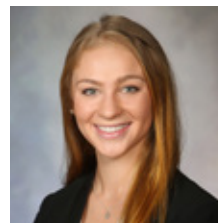
Reese Imhof is originally from Long Island, New York. Prior to medical school, he worked in graphic design and owned his own business. In

his spare time you can find him hanging out with his wife, two cats and dog, running, hiking, or doing photography.



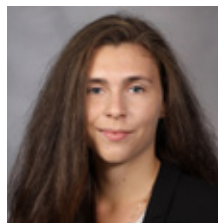
Deeyar Itayem comes from Memphis BBQ brisket and garlicky hummus. She is a fourth year medical student soaking up all that Mayo Clinic and

her inspiring classmates have to offer before embarking on her Otolaryngology residency. A citizen of the world, Deeyar loves cooking up a storm, is convinced that strong is beautiful, and believes that dancing is the best medicine.



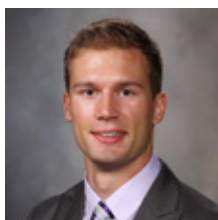
Sydney Larkin is a third year medical student, originally from Cincinnati, Ohio. She recently completed her Masters in Science of Healthcare from

Arizona State University and is pursuing a career in Dermatology. In her spare time, she enjoys running, hiking, playing soccer, and taking photographs.



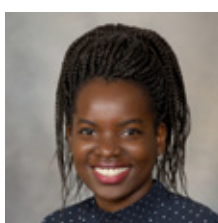
Karina Lenartowicz is a first year medical student with an interest in surgery. In her spare time, she enjoys rock climbing, volleyball, and complaining about

the imperial system of measurement.



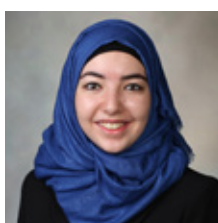
Kevin Miller is a third year medical student from Madison, WI. He is interested in hematology/oncology, and has a particular passion for destroying

malignant plasma cells. He claims no hobbies, save for hanging out with his soon-to-be wife, Nicola.



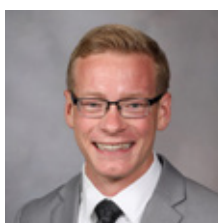
Deborah "Debbie" Msekela, originally from Tanzania, is a second year Ph.D. student in the MPET department. Outside of work, she enjoys

listening to music, reading and binge watching Game of Thrones. Also, thanks to the Mitchell student lounge, she has discovered a new favorite pastime: ping-pong!



Tala Mujahed is a second year medical student. She is from Phoenix, Arizona and she enjoys painting, drawing, and reading in her spare time. She

is super excited to start seeing patients and getting more involved in patient care next year.



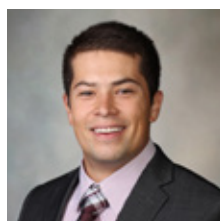
Benjamin Nelson is a first year medical student. Aside from making unrenal Urology puns, you can find him at the gym or splayed out in the

lecture hall eating oatmeal. Unofficially voted the loudest laugh of the first year class.



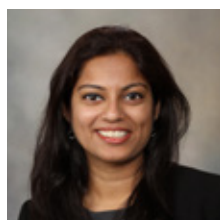
Anjali Panicker is a first year Ph.D. student. She is originally from Mumbai, India, where she earned her engineering degree in Biotechnology. She

then moved halfway across the globe to Chicago to pursue her Master of Science in Medical Biotechnology from UIC. She's now at Mayo to learn more about immunology. Her passions outside of immunology research include fitness, mixed martial arts, cooking, and travelling.



Austin Peña is a second year medical student interested in emergency medicine, family medicine, and surgery. He spends his time drawing, hiking/

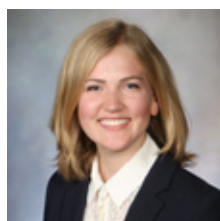
biking, exploring cuisines. He is looking forward to working as a physician and being active in the policy arena.



Ramya Rallabandi is a second year Ph.D. student in the Virology and Gene Therapy department. Originally from India, she graduated

from University of Florida with a Masters in Microbiology and Cell Science.

Outside work, she really enjoys traveling, photography and painting.



Tori Riccelli would be a third year medical student at Mayo, but she decided to take a year off to teach mice how to climb ladders (with lasers!) at Janelia

Research Campus. In her free time, she enjoys writing, making self-deprecating jokes, and creating custom emojis.



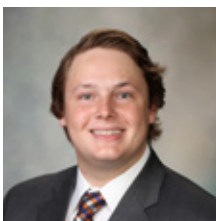
Hiba Saifuddin is a first year medical student from Troy, Michigan with an interest in orthopedics and sports medicine. When she is not

worrying about keeping up with Anki cards, she enjoys drawing, dancing, mixing music, reading, Netflixing, and going to Saturday morning yoga.



Kristen "Kristy" Sessions is a soon-to-be pediatrician who is headed to Lurie Children's Hospital at Northwestern for residency. She

is a native Floridian who gave up her childhood dreams of dolphin training and gator wrestling to move to the wintry north for medical school. Last year, she lived in Lilongwe, Malawi, where she worked as a pediatric pneumonia researcher, and after nine months she finally mastered how to say "researcher" in Chichewa (yofufuza-fufuza). She enjoys road trips, water sports, and new outdoor adventures.



Jon Sussman is a first year medical student raised by the city of Chicago. He has a passion for the history and philosophy of science and intends

on a career in Psychiatry. Jon spends his free time analyzing Illinois-specific political polling and listening to Chance the Rapper on repeat.



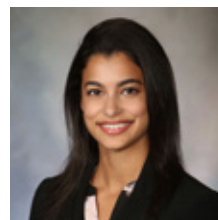
Kekoa Taparra is a third year medical student from Mililani, HI. He completed his undergraduate work in Connecticut at Fairfield University, and then

earned a Ph.D. at Johns Hopkins School of Medicine in Baltimore, MD. He hopes to continue the East to West trend and make it all the way back home to care for members of his indigenous communities. He enjoys spending time with his 'ohana, underwater photography, and swimming.



Jenny Yoon is a first year medical student from New Jersey. She most recently conducted Roman archaeological fieldwork in Italy

and hustled as a paralegal in Washington D.C., but is extremely grateful to be finally studying medicine with her amazing classmates. In her free time, she enjoys writing Yelp reviews, museum-hopping, and practicing yoga.



Stephanie Youssef is a third year medical student interested in ENT surgery. She wrote argumentative essays as a weekly opinions columnist

for the Daily Illini newspaper, but her true passion for writing is in poetry and creative rhetoric. She enjoys biking, snowboarding, Rubik's cubing, and is dedicated to finding the world's most difficult Rubik's cube.



SPRING TRICKLE

BY ANJALI PANICKER